ICU PREDICTIVE SCRIBE PROJECT

AI-Generated Clinical Documentation Examples:

EXAMPLE 1: HIGH RISK PATIENT (MEWS 9)

SOAP NOTE - ICU002
Date/Time: 2024-03-20, 16:00
MEWS Score: 9 (HIGH RISK - Critical Deterioration)

S (Subjective):
Patient reports severe shortness of breath, chest tightness, and feeling "extremely weak." States "I can't catch my breath." Appears distressed and anxious. Family at bedside, expressing concern.
O (Objective):
Vital Signs:

- HR: 125 bpm (↑↑ severe tachycardia)
- SpO2: 88% on room air (↓↓ critical hypoxemia)
- MAP: 58 mmHg (↓↓ severe hypotension)
- RR: 28 breaths/min (↑↑ tachypnea)
- Temp: 38.5°C (↑ fever)

Respiratory: Labored breathing, use of accessory muscles, bilateral crackles on auscultation
Cardiovascular: Tachycardic, weak peripheral pulses, cool extremities, delayed capillary refill >4 sec
Neurological: Alert but anxious, diaphoretic, following commands
A (Assessment):
Acute severe clinical deterioration consistent with septic shock or acute respiratory failure. MEWS 9 indicates life-threatening status requiring immediate medical intervention. Hemodynamic instability with compensatory tachycardia failing to maintain adequate perfusion. High risk for cardiopulmonary arrest.
P (Plan):
IMMEDIATE ACTIONS:

1. **Escalate NOW:** Physician/Intensivist notified STAT - Code Blue team on standby
2. **Airway/Breathing:** High-flow oxygen 15L via non-rebreather mask, prepare for possible intubation, RT consulted
3. **Circulation:** Establish 2nd large-bore IV access, 500mL NS bolus STAT, prepare vasopressor infusion
4. **Diagnostic:** STAT ABG, CBC, lactate, blood cultures x2, chest X-ray portable
5. **Monitoring:** Transfer to ICU Level 1 bed, continuous cardiac monitoring, arterial line placement ordered, Foley catheter for strict I&O

Re-assessment: Q5 minutes until stabilized, continuous SpO2/cardiac monitoring
Documented by: ICU Nurse
Physician notification: Dr. [Name] notified at 16:02, at bedside

EXAMPLE 2: LOW RISK PATIENT (MEWS 2)

SOAP NOTE - ICU001
Date/Time: 2024-03-20, 20:00
MEWS Score: 2 (LOW RISK - Stable)

S (Subjective):
Patient reports feeling "much better" compared to earlier in the day. Denies shortness of breath, chest pain, or discomfort. States appetite is returning. Resting comfortably in bed, conversing appropriately with family.

O (Objective):
Vital Signs:

- HR: 88 bpm (within normal limits)
- SpO2: 95% on room air (acceptable)
- MAP: 78 mmHg (adequate perfusion)
- RR: 18 breaths/min (normal)
- Temp: 37.3°C (afebrile)

Respiratory: Clear breath sounds bilaterally, no accessory muscle use, unlabored respirations
Cardiovascular: Regular rhythm, strong peripheral pulses, warm extremities, capillary refill <2 sec
Neurological: Alert and oriented x3, calm affect, cooperative

A (Assessment):
Hemodynamically stable patient with improving clinical status. MEWS 2 indicates low risk for acute deterioration. All vital signs within acceptable parameters. Patient responding well to current treatment plan. No immediate concerns identified.

P (Plan):
ROUTINE CARE:

1. **Monitoring:** Continue standard Q4H vital signs monitoring
2. **Activity:** Encourage mobility as tolerated, ambulation with assistance
3. **Nutrition:** Advance diet as tolerated, monitor intake
4. **Medications:** Continue current medication regimen per physician orders
5. **Re-assessment:** Next full assessment at 00:00 (midnight shift)

Escalation Criteria: Notify physician if MEWS ≥3, SpO2 <92%, HR >110 or <60, new onset chest pain, or any sudden change in patient condition
Documented by: ICU Nurse
Next Review: 00:00 or PRN with clinical change

KEY OBSERVATIONS:

- High-risk note triggers immediate escalation & interventions
- Low-risk note focuses on routine monitoring & patient comfort
- Both maintain JCI compliance standards
- AI adapts clinical tone based on patient acuity
- Documentation time: 30 seconds per note vs. 10-15 minutes manual
- Generated using: Anthropic Claude AI
- Clinical validation: Based on MEWS scoring protocol
- Compliance: JCI documentation standards